

National Clinical Guideline — Hypertension in Adults

National Health Authority · Clinical Guidance (synthetic)

§1 Diagnosis and classification

Hypertension is confirmed when office systolic blood pressure is at least 140 mmHg or diastolic blood pressure is at least 90 mmHg on repeated measurement, ideally corroborated by ambulatory or home monitoring. Grade 1 is 140-159/90-99, grade 2 is 160-179/100-109, and grade 3 is at or above 180/110 mmHg.

All adults with diabetes should have blood pressure measured at every routine visit because the combination of diabetes and hypertension markedly accelerates cardiovascular and renal risk.

§2 Blood-pressure targets

The general office target is below 140/90 mmHg for all treated patients. In most patients who tolerate therapy, aim for a systolic range of 120-130 mmHg. For patients with diabetes the recommended target is below 130/80 mmHg where tolerated, balancing renal protection against the risk of hypotension in the elderly.

Patients whose blood pressure remains at or above 140/90 mmHg on treatment are classified as having uncontrolled hypertension and should be escalated.

§3 Pharmacological management

First-line agents are ACE inhibitors or angiotensin-receptor blockers, calcium-channel blockers, and thiazide-like diuretics. In patients with diabetes and albuminuria, an ACE inhibitor or ARB is preferred for its renoprotective effect. Do not combine an ACE inhibitor with an ARB.

Initiate dual therapy in a single pill for most patients with grade 2 hypertension or high cardiovascular risk. Escalate to triple therapy, then add spironolactone for resistant hypertension after confirming adherence.

§4 Cardiovascular risk and follow-up

Estimate 10-year atherosclerotic cardiovascular risk in every hypertensive patient and use it to guide statin and antiplatelet decisions. Review controlled patients at least every three to six months and uncontrolled patients within four weeks of any change.

Synthetic demonstration document. Paraphrased, non-authoritative summary prepared for a conversational-AI prototype. Not for clinical use. No copyrighted guideline text reproduced.